POLICY BRIEF

# Malaria Net Programme: Fix Delivery Before Scaling Up

**To:** Hon. Commissioner for Health, State Ministry of Health
**From:** Kingsley Nwaginineme, Data Analyst
**Date:** 12 November 2026
**Basis:** Household survey of 600 households in LGAs A, B and C (200 each)

## 1. Recommendation

**Do not scale the programme statewide yet. Fix it first, then expand.** The programme clearly helps families start using nets, but it is not yet reaching enough households, and the health benefit we can prove is small. I recommend:

- **Fix LGA C first.** Only 47% of households there use a net, against 55% in A and 64.5% in B. Even among households in the programme, LGA C reaches 59%, compared with 73% (A) and 77% (B). Find out why: delivery, distribution points or distance.
- **Bring in the households left out.** Households outside the programme use nets far less (42% against 70%).
- **Then expand with a proper test.** Compare areas that receive the programme with similar areas assigned by chance, so we can be sure of the benefit before committing statewide funds.

## 2. Evidence

- **Net use is below target.** 55.5% of households use a net (likely range 51.5% to 59.5%). The 80% target lies well outside this range, so the gap is real and not a fluke of the sample.
- **The programme raises net use.** 70% of programme households use a net against 42% of others, a 28-point difference.
- **Blood health improves only slightly.** Programme households average haemoglobin of 11.18 g/dL against 10.93, a gain of 0.25 g/dL (likely range 0.003 to 0.49). Statistically this is just detectable (p = 0.047), but the effect is small. Average haemoglobin across all households is 11.05 (likely range 10.93 to 11.17), below the 11.5 benchmark.
- **Early sign of fewer episodes.** Programme households go about 125 days between malaria episodes, against about 88 days for others. This is a supporting pattern only.

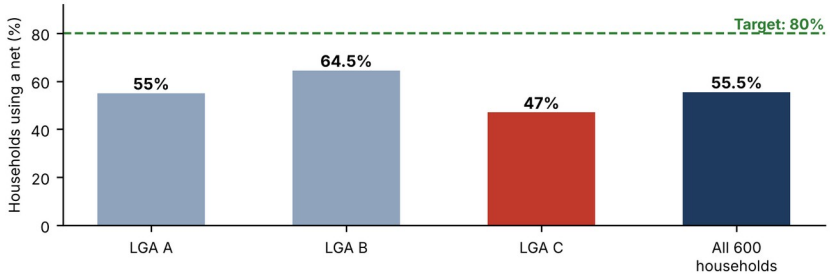



*Figure 1. Share of households using a net, by LGA, against the 80% target.*

## 3. Risk of being wrong

Two mistakes are possible. **Scaling up a programme whose health benefit is not real** would waste statewide funds and give false confidence. **Stopping a programme that does help** would leave families without protection. The haemoglobin result sits right at the edge of significance (p = 0.047), so a small change in the data could flip it. For a decision this large, the first mistake worries me more, which is why I advise fixing and testing before expanding. Stopping the programme is not advised: the rise in net use is large and clear.

## 4. Limits of this analysis

- **Limited coverage.** The data covers 600 households in three LGAs, so results may not hold in every part of the state.
- **No random assignment.** Households were not randomly placed in the programme. Those who joined may differ (for example in distance to clinics), so we see association, not proven cause.

- **Borderline and small effect.** The haemoglobin gain is barely significant and small in practical terms.
- **One-time snapshot.** There are no before-and-after figures, so we cannot say how much the programme changed things over time.

## Decision requested

Approve a targeted fix of delivery and uptake in LGA C, and a properly designed evaluation, before any statewide expansion. I can present the supporting figures at your convenience.